

MEDICAL RECORD -- ORTHOPEDIC SURGERY & PERIOPERATIVE CARE

PATIENT

Patient Name: Aaliyah Monroe
Date of Birth: 2005-07-14
Member / Plan ID: MCD-000003
Date of Service: 2026-06-17
Rendering Provider: Paul Eriksson, MD -- Orthopedic Surgery

CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS

21-year-old patient with severe, end-stage POST-TRAUMATIC osteoarthritis of the RIGHT knee. History of a high-energy tibial plateau fracture 4 years prior (MVA), treated with ORIF, complicated by post-traumatic arthrosis with progressive bone-on-bone degeneration. Despite the young age, the joint is non-salvageable; total knee arthroplasty is indicated for post-traumatic etiology, not primary age-related osteoarthritis.

PRE-OPERATIVE IMAGING

Weight-bearing radiographs of the RIGHT knee (3 views) dated prior to surgery: Tricompartmental joint-space obliteration, subchondral sclerosis and cyst formation, marginal osteophytes, and post-traumatic articular incongruity of the lateral tibial plateau. Kellgren-Lawrence grade 4. Findings consistent with ICD-10 M17.11 (unilateral primary/post-traumatic osteoarthritis, right knee).

FAILED CONSERVATIVE MANAGEMENT (>12 MONTHS)

Documented failure of: structured physical therapy (16 visits over 6 months), two intra-articular corticosteroid injections, a course of viscosupplementation, NSAIDs, activity modification, and an unloader brace. Persistent mechanical symptoms, effusion, and inability to ambulate without assistance.

OPERATIVE REPORT -- CPT 27447

Procedure: Total knee arthroplasty, right (CPT 27447)
Pre-op Diagnosis: Post-traumatic osteoarthritis, right knee (M17.11)
Anesthesia: Spinal with adductor canal block

Standard medial parapatellar approach. Severe tricompartmental degeneration with post-traumatic deformity confirmed intra-operatively. Femoral, tibial, and patellar resurfacing performed; cemented posterior-stabilized implant placed; trial range of motion 0-120 degrees with stable ligamentous balance. EBL minimal. Patient tolerated the procedure well; no complications. Operative note dictated and signed by Dr. Eriksson.

OFFICE EVALUATION & MANAGEMENT -- CPT 99215 (HIGH COMPLEXITY)

High-complexity MDM supporting a level-5 established-patient E/M: TWO chronic illnesses managed -- (1) post-traumatic osteoarthritis with surgical planning, and (2) essential hypertension (I10). Data reviewed: outside ORIF operative records, prior imaging, PT notes, and labs. Risk: decision for major elective surgery with perioperative anticoagulation and anesthesia clearance -- high morbidity risk. Extended counseling on surgical vs non-surgical options and post-op expectations.

HYPERTENSION MANAGEMENT -- ICD-10 I10

Essential hypertension, in-office BP 148/92. Lisinopril increased from 10 mg to 20 mg daily; pre-operative cardiac risk reviewed and optimized. Counseled on diet, home BP monitoring, and medication adherence. Follow-up in 2 weeks.

ASSESSMENT & PLAN

1) M17.11 post-traumatic OA, right knee -- status post total knee arthroplasty (27447); weight-bearing as tolerated, DVT prophylaxis, PT. 2) I10 essential hypertension -- medication titrated, optimized for surgery.

SERVICES DOCUMENTED THIS ENCOUNTER

27447 Total knee arthroplasty, right
99215 Office/outpatient visit, established, high complexity

99213 Office/outpatient visit, established, low/moderate

Date of Service: 2026-06-17

ATTESTATION

I attest that the above documentation is complete and accurate for the services rendered to Aaliyah Monroe (MCD-000003) on 2026-06-17. Electronically signed, Paul Eriksson, MD, Orthopedic Surgery.